



Implementation Scope

Strategic Scope: Aligning clinical care, research (genetics, grants), and data exchange across networks and external partners like radiology and labs.

Integrate core EHR with ancillary systems—including Pharmacy, Clinical Labs, Imaging (PACS), Billing/Revenue Cycle, and Research/Genetics. Enable seamless clinical data sharing across non-acute care transitions and external partners.

**"Safe implementation,
strong governance,
lasting improvement"**

Strategic Priorities

Implementation Focus



Needs Assessment: Identify workflow barriers, system priorities, and subject matter experts from clinical and IT areas.



SME Integration: Identify IT Subject Matter Experts to add to the project management team. Frontline SMEs are critical to ensure real-world clinical workflows and prevent care disruptions. (Moore, 2022)



Legacy Integration: Prioritize high-risk interfaces in clinical labs and imaging. Evaluate API requirements for conversion.



Financial Stewardship: Use \$238M in reserves for infrastructure like IV pumps, medication barcodes, and system hardware.

Governance & Execution



Change Management: Redesign workflows and support adoption.



Staffing & Governance: Review IT staffing and define decision rights.



Command Center: Use daily huddles to track implementation progress.

Why This Plan Works



Leadership

Hire an established CIO to lead the EHR team. (Hütter & Riedl, 2020) Establish an executive steering committee to define decision rights and monitor risk. As we have learned from successful and unsuccessful integrations with Children's Hospital of Pittsburgh (Han et al., 2005) and Kaiser Permanente (Bornstein, S., 2012)



Legacy Systems

Rank systems by upgrade urgency (separating immediate high-risk lab/imaging interfaces from deferred updates) and evaluate API conversion tools to mitigate data migration risks.

Audit existing mainframe capacity, IV pumps, and barcode scanners.